

=====

FACSIMILE TRANSMISSION

TO: Prior Authorization Review

RE: Clinical note – Inpatient admission – COVID-19 isolation with pneumonia and hypoxemia

DATE: 2021-01-03

PAGES: 1

=====

****Subjective:**** Ariane Runolfsson is an 81-year-old woman admitted for isolation with disease caused by SARS-CoV-2. She reports five to six days of initially dry cough followed by persistent fever, anorexia, and progressively worsening dyspnea, now present with minimal exertion – she required rest stops walking from bed to bathroom this morning. An outpatient SARS-CoV-2 test several days ago was positive; a repeat NAA test on arrival is also positive. She describes chest tightness with breathing but explicitly denies chest pain or pressure resembling her prior myocardial infarction. Her daughter, present at admission, reports mild morning foggy but no falls, syncope, hemoptysis, or leg swelling; the patient is conversant and oriented. Poor oral intake for two days. Past history: essential hypertension, acute ST segment elevation myocardial infarction with history of MI, hyperlipidemia, metabolic syndrome X, prediabetes, BMI 30+ obesity, osteoporosis, and adolescent idiopathic scoliosis; history of tubal ligation and past miscarriage. Allergies: tree nut (allergic disposition) – dietary flagged. Home medications confirmed with daughter: aspirin 81 mg, atenolol 50 mg, rosuvastatin calcium 40 mg, and lisinopril 20 mg daily, with good adherence. ROS otherwise negative except as above.

****Objective:**** Admission vitals: T 38.45 C, RR 19.28/min, HR 56.97/min, SpO2 87.18% (arterial), BP 105/53 mmHg, weight 73.3 kg. Repeat vitals with clinical deterioration: T 38.92 C, RR 24.15/min, HR 89.33/min, SpO2 75.7%, BP 97/53 mmHg. Exam: fatigued, speaking in short sentences with increased work of breathing; alert and oriented; findings consistent with pneumonia on auscultation; thoracic curvature consistent with known scoliosis; no peripheral edema. Plain chest x-ray: findings consistent with pneumonia. Initial labs: WBC $3.42 \times 10^3/\mu\text{L}$, hemoglobin 12.5 g/dL, hematocrit 45.39%, platelets $136.34 \times 10^3/\mu\text{L}$, neutrophils $1.6 \times 10^3/\mu\text{L}$, lymphocytes $1.07 \times 10^3/\mu\text{L}$; glucose 67.23 mg/dL, BUN 13.39 mg/dL, creatinine 2.66 mg/dL, GFR (MDRD) 11.1 mL/min, sodium 136.05 mmol/L, potassium 5.07 mmol/L, chloride 101.64 mmol/L, CO2 21.79 mmol/L, calcium 9.76 mg/dL, albumin 5.44 g/dL. SARS-CoV-2 RNA panel positive x2.

****Assessment and Plan:****

COVID-19 with pneumonia

81-year-old with confirmed SARS-CoV-2 infection (two positive NAA panels) and radiographic pneumonia, admitted for isolation given hypoxemia and high-risk comorbidities.

- Hospital admission to isolation room; no in-room visitors, daily family updates and video calls arranged.
- Daily monitoring labs: CBC with automated differential, comprehensive metabolic panel; scheduled troponin I (high-sensitivity), PT/coagulation panel, and iron panel.

Hypoxemia and respiratory distress

SpO2 87.18% on arrival with subsequent desaturation to 75.7%, tachypnea to 24.15 /min, and fever to 38.92 C.

- Oxygen administration by mask, continuous, titrated to saturation; ongoing daily oxygen therapy through the stay.
- Daily prone positioning sessions as tolerated, with padding accommodations for scoliosis.
- Escalation and call-bell instructions reviewed with patient; monitor for worsening respiratory distress.